



SURGICAL PATHOLOGY REPORT
*** Addendum ***

Patient Name:

Accession #:

Med. Rec. #:

Date of Procedure:

DOB: (Age):

Location:

Date of Receipt:

Gender:

M

Service:

Urology

Date of Report:

Ref. Physician:

Account #:

Patient Address:

Billing Type:

Additional Copy to:

Ref. Source:

Clinical Diagnosis & History:

Hx HGT1 + CIS, s/p BCG; now with (+) cytology

Specimens Submitted:

1: Left lateral bladder wall

DIAGNOSIS:

1. Left lateral bladder wall:

Tumor Type:

Invasive urothelial carcinoma with focal glandular and basaloid/squamoid features

Histologic Grade:

High grade

Pattern of growth of the non-invasive component:

No in situ carcinoma

Local Invasion:

Muscularis propria

Vascular Invasion:

Not identified

Pw TSS, TCGA tumor is pure urothelial

ICD-O-3

Carcinoma, urothelial NOS 8120/3

Site: Bladder, lateral wall C67.2

Q10 4/30/14

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Procedures/Addenda:

Addendum

Date Ordered:

Status: Signed Out

Date Complete:

Date Reported:

Addendum Diagnosis

1. Left lateral bladder wall:

The tumor has mixed histologic features. By immunostains, staining heterogeneity was present in the following markers: CK7, CK20, GATA3, HMWK (34be12), p63/4A4 and CD117. Strong staining with p16 and focal synaptophysin was present. The tumor was negative for chromogranin, PAX8, racemase, calponin, ERG and S100. There were focal areas of faint PSA and PSMA labeling, which might suggest prostatic differentiation, but their true significance is not clear as the tumor is showing more convincing differentiation along the urothelial phenotype. This case was discussed at the

Some of the immunohistochemistry and ISH tests were developed and their performance characteristics were determined by the Department of Pathology. They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under Clinical Laboratory Improvement Amendments of 1988 (CLIA '88) as qualified to perform high complexity clinical laboratory testing.

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*** Report Electronically Signed Out ***
Signed out by

Gross Description:

1) The specimen is received in labeled "left lateral bladder wall", and consists of multiple pieces of tan soft tissue measuring 2.5 x 2.5 x 1.5 cm in aggregate. The specimen is entirely submitted in three cassettes. Summary of sections:

Summary of Sections:

Part 1: Left lateral bladder wall

Blocks	Block Designation	PCs
3		27

Special Studies:

Result	Special Stain	Comment
	CK7	
	4A4/P63	
	CK20	
	34BE12	
	PSA	
	PSMA	
	P16.	
	SYN	
	CHR	
	PAX8.	
	VIMEN	
	IMM RECUT	
	NEGATIVE.	
	RACEMASE	
	ERG.	
	HPV III FAMILY	
	CD117	
	S100	
	CALP	
	PSA	

BCG > 90 days
to tumor
procurement.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	///	Date Reviewed 2/3/13